

Credentialing Management Portal

Business Requirements Document (BRD) and Functional Specification For Revantage Healthcare Business Solutions LLC

Prepared For	Revantage Healthcare Business Solutions
Prepared By	Internal Requirements Draft for Software Development Team
Version	Version 2.0
Date	April 1, 2026

Purpose of this document: define the functional, operational, and technical expectations for a web-based credentialing portal with SQL Server backend, email integration, task tracking, delay attribution, document management, and reporting.

1. Business Need

Revantage currently manages provider credentialing through spreadsheets, email attachments, and manually maintained server folders. This approach does not provide a reliable single source of truth, makes follow-up history difficult to track, creates risk when documents expire or staff change, and makes it hard to clearly separate internal team delay from provider-side delay.

The proposed portal should function as a centralized credentialing operations platform rather than only a document repository. It should track provider data, payer applications, document requests, team tasks, status history, email communication, and aging metrics in one place.

2. Business Objectives

- Replace spreadsheet-driven credentialing tracking with a structured web application and SQL Server database.
- Provide one unified record for each provider, practice, location, payer enrollment, and supporting document.
- Track credentialing, enrollment, revalidation, and maintenance work payer-wise and state-wise.

- Integrate **credentialing@revantagehbs.com** with the portal so that email activity becomes part of the workflow.
- Automatically follow up by email when inquiries or document requests are pending and no response is received within configured timelines.
- Separate delay ownership between Revantage team delay, provider/practice delay, payer delay, and system hold status.
- Create management visibility through dashboards, SLA aging, recredentialing reminders, and workload reporting.

3. Scope

3.1 In Scope

- Web portal for internal credentialing team users, managers, and administrators.
- Optional secure external access for providers or practice contacts to upload requested documents.
- SQL Server data store for master data, workflow records, tasks, documents metadata, and audit logs.
- Email integration for inbound and outbound email handling through **credentialing@revantagehbs.com**.
- Document repository, task engine, reminders, dashboards, status tracking, and reporting.

3.2 Out of Scope for Phase 1

- Full payer portal automation or robotic submission to all payer websites.
 - *I am not sure how doable it is, but it looks like API set up might be needed. For now, this is just a concept item - Ronnie*
- Direct EHR/PMS integration unless separately approved.
- Electronic signature workflows beyond simple attestation unless later expanded.

4. User Roles

Role	Primary Permissions
System Administrator	Manage users, roles, masters, status definitions, SLA rules, templates, and system configuration.
Credentialing Manager	View all work, assign cases, monitor delay ownership, approve escalations, and review dashboards.
Credentialing Executive	Create and update provider applications, request documents, send follow-ups, and maintain notes.
Billing/Operations Read Only	View provider readiness, effective dates, enrollment status, and key notes.

Role	Primary Permissions
Provider / Practice User (Optional External)	Upload requested documents, review outstanding items, and respond to information requests.

5. Functional Requirements

5.1 Master Data Management

- Practice master: legal name, DBA, EIN/TIN, group NPI, taxonomy, addresses, contacts, specialties, and associated locations.
- Provider master: demographics, NPI, taxonomy, specialty, licenses, DEA/CDS, board certification, CAQH, PECOS, malpractice, work history, and state mapping.
- Payer master: payer name, state applicability, application type, submission channel, portal details, fax, email, phone, turnaround expectations, and participation rules.
- Location/facility master and provider-location-practice relationships.
- Configurable document type master and status master.

5.2 Credentialing Case and Application Management

- Each payer application must be stored as an individual case linked to provider, practice, location, state, and payer.
- The system must capture owner, priority, intake date, submission date, payer follow-up date, expected completion date, effective date, termination date, and revalidation due date where applicable.
- A case may include enrollment sub-types such as credentialing, participation, reassignment, EFT, ERA, roster enrollment, group enrollment, or recredentialing.
- Multiple applications for the same provider must be supported without data duplication.

5.3 Status Management

Statuses should not be hard-coded. The administrator should be able to activate, deactivate, reorder, and map statuses to dashboard categories, SLA rules, and delay attribution logic.

Suggested Status	Meaning / Usage
Not Started	Case created but no action taken yet.

Suggested Status	Meaning / Usage
Pending Internal Review	Revantage team reviewing provider data before requesting or submitting.
Documents Requested from Provider	Information or documents requested from provider/practice; provider response awaited.
Received - Under Review	Provider has submitted information; team validating completeness.
Ready to File	All required items available and application can be filed.
Application Filed / Submitted	Application submitted to payer or via portal/fax/email.
At Payer	Submission acknowledged and pending payer review.
Additional Information Requested by Payer	Payer requested clarification or additional documents.
Escalated to Payer	Formal escalation raised due to delay, issue, or missing response.
Follow-up in Progress	Revantage team actively following up with payer.
Approved	Application approved; effective date captured.
Rejected / Denied	Payer declined application.
Panel Closed	Payer panel closed or not accepting new participation.
Not Eligible	Provider/practice does not meet payer requirements.
On Hold	Case paused for a valid reason.
Terminated	Participation terminated.
Revalidation / Recredentialing Due	Future maintenance activity due or triggered.

5.4 Delay Attribution and SLA Tracking

A critical business requirement is to identify whether a delay belongs to the credentialing team, the provider/practice, or the payer. This must not be left to manual interpretation only.

- Each case must have a current delay owner: Revantage Team, Provider/Practice, Payer, or No Delay / Closed.
- The system must calculate aging separately for total case age and current waiting age.

- When a document request is sent to the provider and no response is received within the configured timeline, delay ownership must automatically move to Provider/Practice.
- When the provider responds and the Revantage team does not act within the defined internal SLA, delay ownership must move to Revantage Team.
- When the application is submitted and payer action is pending, delay ownership must move to Payer unless a team follow-up SLA is missed.
- Managers must be able to see delay aging by owner, executive, payer, provider, and practice.
- Manual override should be allowed for exceptional cases, but each override must be logged with reason and user name.

Suggested SLA model: internal review within 2 business days of receipt, provider reminder after 3 business days of no response, second reminder after 7 business days, manager escalation after 10 business days, and payer follow-up every 5 to 7 business days after submission unless payer-specific rules differ.

5.5 Document Management

- Central repository for provider, practice, and payer-specific documents.
- Document classification by type, provider, practice, payer, state, effective date, and expiry date.
- Version history and ability to keep superseded documents without losing the audit trail.
- Support upload from portal UI and capture from email attachments where business rules allow.
- Document alerts for upcoming expiry such as license, DEA, malpractice, board certification, W-9, voided check, CAQH attestation, and identification documents.

5.6 Email Integration with credentialing@revantagehbs.com

The portal must integrate with credentialing@revantagehbs.com so the email account becomes an operational inbox inside the credentialing workflow rather than a separate mailbox.

- Inbound email sync: read incoming emails, subject, sender, body preview, attachments, received date/time, and thread reference.
- Outbound email sync: emails sent from the portal must be sent through the credentialing mailbox and logged against the correct provider case.
- Thread linking: emails should be associated with the related provider/case either by case ID, mapped email address, or manual linking by staff.
- Attachment ingestion: inbound attachments should be available for review and optional save into the document repository.
- Template emails: document request, reminder, follow-up, escalation, approval notice, revalidation reminder, and internal note alerts.
- Auto-follow-up rules: if an inquiry or document request email is sent and no reply is received within the configured number of business days, the system should automatically send a reminder email.
- Escalation rules: after defined reminder attempts, the case should move into escalation queue and notify manager.
- Mailbox triage view: unlinked emails, unresolved threads, failed sends, bounced emails, and unread priority items must be visible to team leads.
- All automated emails must be configurable and switchable by template, payer, provider group, or case type.

5.7 Activity Log and Communication History

- Every case must maintain a chronological log of calls, payer portal checks, emails, faxes, notes, reminders, escalations, and status changes.
- Each activity should store date/time, activity type, actor, summary, reference number, attachment link if any, and next action date.
- The log must capture both human actions and system-generated actions such as automated reminders or SLA recalculations.

5.8 Task Management and Work Queues

- Tasks shall be created manually as well by add New Task Button
- Cases should generate tasks for missing documents, follow-up due, resubmission, revalidation, approval posting, and document expiry.
- Executives should have a daily work queue showing today due, overdue, waiting on provider, waiting on payer, and escalated items.
- Managers should be able to reassign tasks and redistribute workloads.

5.9 Dashboard and Reporting

- Open applications by status, payer, provider, practice, state, and owner.
- Delay dashboard by team delay, provider delay, payer delay, and total aging.
- Productivity dashboard by executive: cases handled, follow-ups completed, approvals achieved, overdue tasks, and average turnaround time.
- Document compliance dashboard: missing items, expiring items, overdue provider submissions.
- Approval and denial trends, panel closed counts, and recredentialing upcoming.
- Email operations dashboard: pending replies, automated reminders sent, bounced emails, unlinked emails, and manual intervention count.

5.10 External Provider / Practice Portal (Optional but Recommended)

- Secure login for provider or practice users.
- Outstanding document requests and checklist visibility.
- Upload center with accepted formats and size limits.
- View request history and due dates.
- View current high-level status of their payer applications.

6. Non-Functional Requirements

- Web-based responsive application for desktop-first usage.
- SQL Server backend with normalized transactional schema and reporting-friendly views.
- Role-based access control and least-privilege design.
- Full audit trail for login, status change, edit, delete, upload, download, and email actions.
- Search performance should support fast lookup by provider, NPI, payer, tax ID, CAQH, practice, or status.

- System should support configurable business calendars for business-day SLA calculations.
- Document storage should support antivirus scanning and file type validation.
- Application should support backup, restore, and archival strategy.

7. Security and Compliance Requirements

- Secure authentication with password policy and optional MFA.
- Encrypted data in transit and appropriate encryption at rest for sensitive records and documents.
- Granular document permissions where required.
- Immutable audit logging for material events.
- Retention controls for email logs, documents, and case history.

8. Recommended Data Entities

Users, Roles, Practices, Practice Contacts, Locations, Providers, Provider Licenses, Provider Certifications, Provider Malpractice, Provider-Payer Cases, Case Types, Status Master, Delay Owner Master, Tasks, Activities, Email Threads, Email Messages, Document Types, Documents, Document Versions, Payers, Payer Contacts, Escalations, SLA Rules, Notification Templates, Audit Logs.

9. Suggested Phase-Wise Delivery

Phase	Priority	Inclusions
1	Must Have	Master data, case tracking, documents metadata, status engine, tasks, dashboards, audit trail.
2	High	Email integration, auto-reminders, delay ownership logic, escalation rules, richer reporting.
3	Recommended	External provider portal, advanced analytics, template management, bulk imports, API readiness.

10. Acceptance Criteria Summary

- The team can create and track provider credentialing cases without using external spreadsheets.
- A manager can tell at any time which items are delayed by provider, payer, or internal team.
- Emails from credentialing@revantagehbs.com are visible within the related case history.
- Automated reminder emails can be triggered when no response is received within configured timelines.

- Document expiry and revalidation reminders are generated without manual diary tracking.
- Dashboard reports support operational review, accountability, and management decisions.

11. Notes for Software Development Team

Build the system as a configurable workflow platform, not as a rigid one-off screen set. Statuses, SLA values, email templates, document requirements, and reminder rules will evolve over time. The database and application design should therefore separate master configuration from transaction records.

Email integration should support enterprise-safe implementation through Microsoft 365 or approved protocols. Automated reminders should avoid duplicate sends and must respect closed cases, manual overrides, and exception flags.

Delay ownership is a core differentiator of this portal. It should be designed as first-class business logic and not as an afterthought in reporting.

UI Structure | Design

1. Overall UI Structure (Navigation)

Top Navigation Bar

- Logo (Revantage)
- Global Search (Provider / NPI / Payer / Application ID)
- Notifications (alerts, expiring docs, follow-ups)
- Email Inbox  (credentialing@revantagehbs.com integration)
- User Profile

Left Sidebar (Primary Modules)

-  Dashboard
-  Providers
-  Practices
-  Credentialing Tracker
-  Documents
-  Email Center
-  Tasks & Follow-ups
-  Reports
-  Admin / Settings

2. HOME / DASHBOARD (Most Important Screen)

This should immediately answer:

-  What needs attention today?
-  Where is revenue stuck due to credentialing?

Layout

◆ Top KPI Tiles

- Total Active Providers
- Applications in Progress
- Pending with Payer
- Pending with Provider
- Overdue Follow-ups
- Expiring Licenses (next 30 days)

◆ Work Queue (Center Panel)

Today's Action Items:

- 12 Follow-ups due
- 8 Applications stuck > 15 days
- 5 Providers pending documents

Clickable → takes to filtered list

◆ **Delay Ownership Widget**

Delays Breakdown:

- Provider Delay: 32%
- Internal Delay: 18%
- Payer Delay: 50%

👉 This is very powerful for management visibility

◆ **Recent Activity Feed**

- “Aetna application submitted for Dr. Smith”
- “Documents requested from provider – no response 3 days”
- “Follow-up call logged – Molina”

3. PROVIDER MODULE

Provider List View

Table with:

- Provider Name
- NPI
- Specialty
- Practice

- Credentialing Status (overall)
- Pending Payers count
- Expiry Alerts

Search + Filters:

- By State
- By Payer
- By Status
- By Missing Documents

Provider Profile Page (VERY IMPORTANT)

Tabs inside provider:

Overview

- Basic details
- Linked practices
- CAQH, NPI, DEA, licenses

Credentialing (Core Tab)

Grid:

Payer | Status | Submitted Date | Last Follow-up | Owner | Delay Type

Click → opens full application detail

Documents

- License
- DEA
- Insurance
- W9
- CV
- Each with expiry + version history

Communication

- All emails (auto logged from inbox)
- Notes
- Calls

Timeline

Full audit trail:

- Status changes
- Emails sent
- Docs uploaded

4. CREDENTIALING TRACKER (Core Engine)

This is your **main working screen for team**.

Table View

Columns:

- Provider
- Payer
- Status (dropdown)
- Assigned To
- Last Action Date
- Next Follow-up Date
- Delay Type (Auto-calculated)
- Aging (days since submission)

Filters (Very Important)

- Pending with Provider
- Pending with Payer
- Overdue
- Recently Submitted
- Escalated Cases

Status Dropdown (Configurable)

Examples:

- Not Started
- Documents Requested from Provider

- Documents Received
- Application Filed
- Submitted to Payer
- In Process (Payer)
- Additional Info Requested
- Escalated to Payer
- Approved
- Rejected
- Panel Closed
- Not Eligible

Application Detail Drawer (Side Panel)

When clicked:

- Full status timeline
- Document checklist
- Notes
- Email history
- Follow-up log
- Attachments

5. EMAIL CENTER (Game Changer Feature)

This will differentiate you.

Inbox View

- Sync with credentialing@revantagehbs.com
- Thread-based conversations
- Tag by:
 - Provider
 - Payer
 - Application

Smart Features

Auto Tagging

System reads subject/body:

→ links email to provider + payer

Follow-up Automation

Example:

- Email sent → no reply in 3 days → auto reminder
- No reply in 7 days → escalation email

Email Templates

- Document request
- Follow-up with payer

- Escalation email

6. TASKS & FOLLOW-UPS

Task Board (Kanban Style)

Columns:

- Due Today
- Upcoming
- Overdue
- Completed

Auto-generated Tasks

- Follow-up due
- Missing documents
- Expiring licenses
- Revalidation due

Task Detail

- Linked provider + payer
- Notes
- Attachments

- Email trigger button

◆ 7. DOCUMENT MANAGEMENT

Folder Structure (Auto-organized)

Provider → Document Type → Version

Features:

- Upload + drag-drop
- Expiry tracking
- Alerts before expiry
- Mandatory document checklist

👉 This is critical because credentialing heavily depends on documents

◆ 8. DELAY TRACKING LOGIC (VERY IMPORTANT)

System should auto-tag delay:

Rules:

- If status = "Documents Requested" → Delay = Provider
- If no action by team for X days → Delay = Internal
- If submitted but no payer response → Delay = Payer

UI Display:

Color-coded:

-  Internal Delay
-  Provider Delay
-  Payer Delay

◆ 9. REPORTS & ANALYTICS

Key Reports:

- Turnaround time by payer
- Approval rate
- Delay breakdown
- Team productivity
- Applications aging

◆ 10. ADMIN SETTINGS

- Add payers
- Define statuses
- Define document types
- Email templates
- SLA rules (follow-up days)

◆ 11. DESIGN STYLE (IMPORTANT)

Keep it:

- Clean (like CRM tools)
- Data-heavy but readable
- Minimal clicks

Color Suggestions:

- Blue → Primary actions
- Orange → Pending
- Green → Approved
- Red → Delays / Overdue

◆ **12. PHASE-WISE BUILD (Very Practical)**

Phase 1 (Must Have)

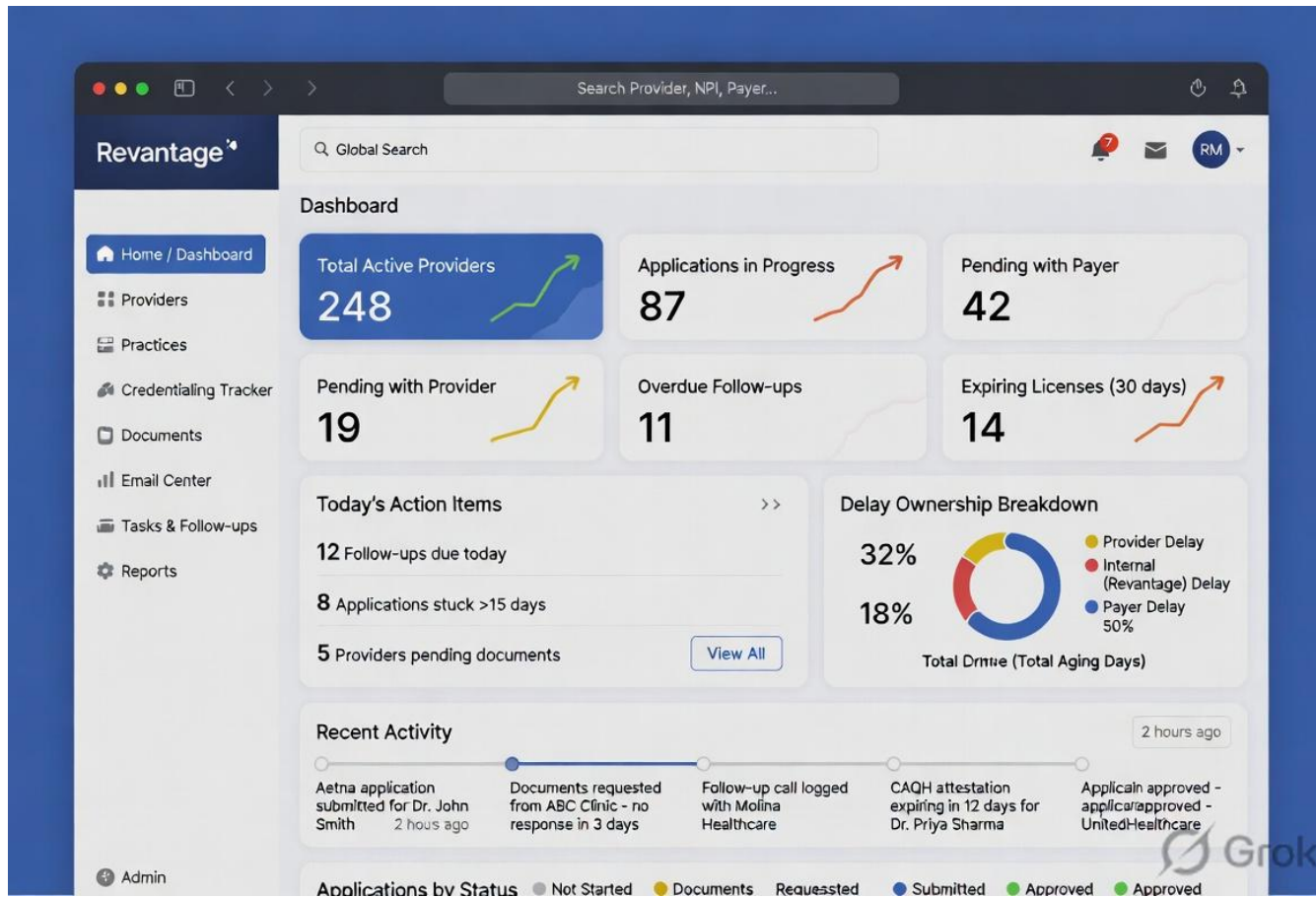
- Provider + Payer + Tracker
- Status + Tasks
- Document storage
- Basic dashboard

Phase 2

- Email integration
- Auto reminders
- Delay analytics

Phase 3

- Provider portal
- Advanced reporting
- AI tagging





Dr. Priya Sharma

NPI 389437 Specialty

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Profile

Information

Basic Information

Demographics

CAQH ID

DEA Number

Licenses Summary

Overview

Credentialing (active)

Documents

Communication

Timeline

...

Profile

Timeline

Payer Name	Application Type	Status	Submitted date	Last Follow-up	Delay Owner	Aging Days
Payer Name	Application Type		6.59.1004	Last Follow-up		49.661
Payer Name	Application Type		6.067814	Last Follow-up		59.450
Payer Name	Application Type		9.506200	Last Follow-up		60.467
Payer Name	Application Type		2.03.1348	Last Follow-up		58.496
Payer Name	Application Type		2.095255	Last Follow-up		60.424
Payer Name	Application Type		1.95.4749	Last Follow-up		41.486
Payer Name	Application Type		2.565293	Last Follow-up		31.486
Payer Name	Application Type		2.563.019	Last Follow-up		12.694

Total Applications: 8

Approved: 6

Pending: 2



Revantage

Search

1

5



Hearn

Secitole

Providers

Autlacs

Providers

Statuss

Credentialing

Providers

Search

Advanced Filter

State

Specialty

Payer

Status

Practice

Provider Name	NPI	Specialty	Practice / Group	State	Overall Credentialing Status	Pending Payers	Expiring Soon	Last Updated
 Dr. John Smith	1234567890	Cardiology	ABC Medical Group	CA	Fully Credentialed		days left	30 days
 Dr. John Smith	1234567890	Cardiology	ABC Medical Group	CA	Fully Credentialed		days left	30 days

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Practices

[Add Practice](#)

Filter [Aspors](#)

By State

AyGage

Group NPI

Group

TIN/EIN

Antup

Status

Antup

Practice Legal Name / DBA	TIN / EIN	Group NPI	Total Providers	Total Locations	Credentialing Status (overall)	Pending Applications	Active Payers
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	5	Blue Cross Blue Shield	
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	3	Blue Cross Blue Shield	
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	3	Blue Cross Blue Shield	
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	3	Blue Cross Blue Shield	
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	3	Blue Cross Blue Shield	
ABC Healthcare Group	12-3456789	9876543210	18	In Progress	3	Blue Cross Blue Shield	

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Add Application
Request Document
Send Email
Add Task

Dashboard
Providers
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Applications
Tasks
Documents
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Reports
Admin
Settings

Jayne Loom, MD
Active

Radiology / Imaging

NPI 1234567890 | CAQH ID 1456789

Primary Practice
John Hopkins Imaging

Primary State
MD

Overall Status
In Progress

Current Delay Owner
Payer Delay

Assigned To
Sarah Wilson
Credentiaing Executive

Next Follow-up
May 16, 2025
In 3 days

Total Payer Applications	Approved	In Progress	Pending w/ Provider	Pending w/ Payer	Overdue follow-ups
12	3	6	1	2	2

Provider Snapshot

Date of Birth
06/12/1978

Gender
Female

Primary Phone
(410) 555-2296

Primary Email
jloom.md@gmail.com

Home Address
1012 James Street,
Baltimore, MD 21202

View More

Critical Alerts

⚠️ DEA License expires in 15 days

⚠️ Malpractice expires in 30 days

⚠️ 2 Payer applications overdue

⚠️ CAQH re-attestation due in 25 days

Recent Activity

06/12/2025
Application submitted to UHC

06/10/2025
Email sent to BCBS - Document Request

05/08/2025
License uploaded: MD State License

05/07/2025
Task completed: Follow up with Aetna

Overview
Practice & IDs
Licenses
Documents
Payer Applications
Communication
Tasks
Delay Tracking
Timeline
Billing Readiness

Back to Provider Overview Practice & IDs Licenses Documents Pager Applications Communication More ▾ 	
Edit	
Practice Information	
Practice Legal Name	John Hopkins Imaging
DBA Name	John Hopkins Imaging
TIN / EIN	52-1234567
Group NPI	1212334455
Practice Type	Group
Billing Address	600 N Wolfe St, Baltimore, MD 21207
Pay-to Address	Same as Billing
Practice Contact	Karen Adams
Contact Email	karen.adams@jhimaging.com
Contact Phone	(410) 955-8800
Provider Identification	
Individual NPI	1234567890
NPI Type	Individual
Taxonomy Code	2085A0202XX
CAGH ID	1456789
CAGH Attestation Date	09/15/2025
CAGH Re-attestation Due	07/18/2025
PECOS Status	Active
Medicare PTAN	12345678
Medicals ID (MD)	987654321
URN#	-
Provider Demographics	
Date of Birth	06/12/1978
Gender	Female
SSN (Last 4)	***-**-7890
Primary Email	jloom.md@gmail.com
Primary Phone	(410) 555-3388
Home Address	1012 James Street, Baltimore, MD 21202
Mailing Address	Same as Home Address
Provider Professional Details	
Provider Type	Physician
Specialty	Radiology
Sub-specialty	Diagnostic Imaging
Board Certification	American Board of Radiology
Employment Type	Employee
Provider Start Date	01/01/2021

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Overview	Practice & IDs	Licenses	Documents	Payer Applications	Communication	More ▾
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Licenses & Certifications

[+ Add License](#)

Type	State	Number	Issue Date	Expiry Date	Status	Days Left	Verified On
Medical License	MD	D123456	03/15/2016	05/30/2025	Active	46	06/01/2024
DEA License	MD	AB1234567	01/01/2022	05/25/2025	Active	115	06/01/2024
COS License	MD	CS123456	07/01/2022	05/30/2025	Active	115	06/01/2024
Board Certification	ABR	12345	07/01/2015	06/30/2030	Active	1686	06/01/2024
Malpractice Insurance	XYZ Insurance	MPL987654	01/01/2024	06/30/2025	Active	411	06/01/2024
OIG Check	-	-	04/10/2025	-	Clear	-	04/10/2025
SAM Check	-	-	04/10/2025	-	Clear	-	04/10/2025
NPDB Check	-	-	04/10/2025	-	Clear	-	04/10/2025
Medicare Opt-Out	-	-	-	-	No	-	-

Overview	Practice & IDs	Licenses	Documents	Payer Applications	Communication	More ▾	Upload Document	
Provider Documents								
Document Type	Status	Expiry Date	Days Left	Version	Required For	Uploaded By	Uploaded On	Actions
State License	Verified	05/30/2026	46	1.0	All	Sarah Wilson	06/01/2025	
DEA License	Verified	06/23/2025	15	1.0	All	Sarah Wilson	06/01/2025	
Malpractice Insurance	Verified	06/30/2025	411	1.0	All	Sarah Wilson	05/01/2025	
CV	Verified	-	-	1.0	All	Sarah Wilson	04/18/2025	
W-9 / Resume	Verified	-	-	1.0	UHC, BCBS, Aetna	Michael Brown	05/02/2025	
CAGH Attestation Proof	Verified	07/15/2025	62	1.0	All Commercial	Sarah Wilson	01/18/2025	
Board Certification	Verified	06/30/2030	1886	1.0	All	Sarah Wilson	06/01/2024	
Voided Check	Pending	-	-	1.0	EFT	-	-	
EFT Form	Pending	-	-	1.0	EFT	-	-	
Hospital Privileges	Missing	-	-	-	Some Payers	-	-	

← Back Overview	Practice & IDs	Licenses	Documents	Payer Applications	Communication	More ▾	+ Add Payer Application	
Payer Applications								
Payer	Type	Status	Delay Owner	Next Follow-up	Aging (Days)	Eff. Date	Provider ID	Group ID
Aetna	Commercial	Ready to File	Revenage	05/14/2025	3	-	-	-
UnitedHealthcare	Commercial	Additional Info Requested	Payer	05/16/2025	14	-	-	-
BCBS of MD	Commercial	Application Submitted	Payer	05/22/2025	11	-	-	-
Cigna	Commercial	Documents Requested	Provider	05/15/2025	5	-	-	-
Humana	Commercial	Not Started	Revenage	-	5	-	-	-
Medicare (Part B)	Medicare	At Payer	Payer	05/09/2025	28	-	-	-
Medicare Reassignment	Medicare	Not Started	Revenage	-	-	-	-	-

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[Faxes](#)
[Notes](#)

Date	From / To	Subject	Linked To	Type	Status	Actions
05/12/2025	To: UHC	Follow up - Additional Information	UHC	Sent	Unresolved	📧 📎
05/10/2025	From: BCBS	Document Request Letter	BCBS	Received	Unresolved	📧 📎
05/06/2025	To: Provider	License Expiration Reminder	All	Sent	Resolved	📧 📎
05/07/2025	From: Aetna	Portal Access Information	Aetna	Received	Resolved	📧 📎
05/05/2025	To: Medicare	#051 Application Submission	Medicare	Sent	Resolved	📧 📎

[View All Emails](#)

Back Preview	Practice & iOs	Licenses	Documents	Payer Applications	Communication	More
Task	Linked To	Assigned To	Priority	Due Date	Status	
Follow up with UHC for documents	UHC	Sarah Wilson	High	05/16/2025	In Progress	
Upload voided check	Medicare	Sarah Wilson	High	05/14/2025	In Progress	
Review BCBS additional info	BCBS of MD	Michael Brown	High	05/15/2025	In Progress	
Submit Aetna application	Aetna	Michael Brown	Medium	05/20/2025	Not Started	
Verify CAQH information	All Commercial	John Smith	Medium	05/18/2025	Not Started	
Call Cigna payer rep	Cigna	John Smith	Low	05/22/2025	Not Started	
View All Tasks						

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[More](#)
[Override Delay](#)

Current Delay

Delay Owner	Payer
Reason	Payer not responding to submission
Delay Start Date	04/04/2025
Delay Aging	14 Days
SLA Status	At Risk
Last Action	Follow up email sent to payer
Next Action	Call payer and escalate if no response by 05/16/2025

Delay History

From Date	To Date	Delay Owner	Reason
04/04/2025	-	Payer	Payer not responding
04/10/2025	04/23/2025	Provider	Waiting for DEA copy
03/28/2025	04/08/2025	Revantage	Internal review pending

Back to Provider		Practice & Os	Documents	Payer Applications	Communication	<u>Timeline</u>	More ▾	All Events ▾
OH	May 12, 2025	01:15 PM	Application submitted to UHC			Sarah Wilson	Application	
✉	May 10, 2025	10:30 AM	Email sent to BODS - Document Request			Sarah Wilson	Email	
📄	May 06, 2025	09:20 AM	License uploaded: MD State License			Michael Brown	Document	
📄	May 07, 2025	04:45 PM	Task completed: Follow up with Aetna			Michael Brown	Task	
📄	May 06, 2025	02:10 PM	DEA License uploaded			Sarah Wilson	Document	
📄	Apr 30, 2025	10:00 AM	Provider created in system			System	System	

[View Full Timeline](#)

Provider is ready to bill for approved payers.	Payer	Status	Effective Date	Provider ID	Group ID	EFT	ERA	Approval Letter	Billing Notified	Billing Notes
	UnitedHealthcare	In Progress	-	-	-	Pending	Pending	No	No	-
	BCBS of MD	In Progress	-	-	-	Pending	Pending	No	No	-
	Aetna	Ready to File	-	-	-	No	No	No	No	-
	Medicare (Part B)	Approved	05/15/2025	123456789A	1122334455	Yes	Yes	Yes	Yes	For Medicare - EFT and ERA setup required after approval.
	Cigna	Documents Requested	-	-	-	No	No	No	No	Awaiting documents from provider.

Payer forms to be uploaded to the software

There should be the ability to create/download the credentialing packet – to be downloaded in a zip folder, that can be faxed/emailed.